

Induction of Labour and Management of Term Pre-Labour Rupture of Membranes guideline (GL861)

Approval

Approval Group	Job Title, Chair of Committee	Date
Maternity & Children's Services Clinical Governance Committee	Chair, Maternity Clinical Governance Committee	14 th February 2025

Change History

Version	Date	Author, job title	Reason
6.6	July 2024	B Chohan, Consultant O&G	Live change to pg. 3 (1.2 Indications) to include criteria of RFM from 38+6wks
6.7	Nov 2024	S McKenzie, Ward mngnr for Marsh & IOL L4 G Roberts L4 Lead MW	Biennial review – no changes to date
6.8	Apr 2025	S Bailey, HoM	Live change to include reference to NHS England -South East Maternity and Neonatal Team. 2024 Induction of Labour in relation to 1.6
6.9	Feb 2026	S Geddes, Joint AN Services manager	Live change to pg. 12 to add criteria for process to follow if augmentation unable to commence within 24hrs due to capacity
6.10	May 2026	S Wong, Consultant O&G	(pg 6) 1.5.2 - criteria amended to reflect Dinoprostone should <u>NOT</u> be used on history of previous CS service users (pg 7) - removed reference to uterine scar (pg 18) - removed reference to Cook® balloon replaced as uterine tamponade

To be read in conjunction with:

- [Diabetes in pregnancy GL983](#)
- [Hypertension in pregnancy GL952](#)
- [Planning place of birth GL887](#)
- [Intrauterine fetal death GL862](#)

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INDUCTION OF LABOUR GUIDELINE

1.1 Introduction

Induction of labour is indicated for either pregnancy complications or prolonged pregnancy. See *NICE guidance CG190/CG70 for further information*.

1.2 Indications:

See [Appendix 3](#) for RBFT IOL indications and priority.

- Post-maturity (40+7)
- Reduced fetal movements if presenting from 38+6 weeks
- Maternal age ≥ 40 at conception (T) (C)
 - Age 40-44 recommend IOL 40 weeks
 - Age ≥ 45 recommend IOL 38 weeks
- Diabetes (see [Diabetes in pregnancy GL983](#))
- Hypertension/Pre-eclampsia (see [Hypertension in pregnancy GL952](#))
- Fetal growth restriction (with consultant agreement only)
- Intrauterine fetal death (see [Intrauterine fetal death GL862](#))

1.3 Contra-indications:

- Severe fetal growth restriction with confirmed fetal compromise
- Any case of fetal compromise already identified

1.4 Decision for induction

Women should be encouraged to explore, discuss and document their preferences and choices concerning induction of labour early in their pregnancy. This should be recorded within their personalised care plan

Women should be signposted to the induction of labour maternity information leaflet for information to support these discussions

A woman's decision to decline or delay induction of labour after discussion of risks, benefits and alternatives should be respected in all situations

1.5 Process of Induction of Labour – see IOL pathway flowchart ([Appendix 1](#))

- Inductions of labour will be ordered on EPR by community midwives or in ANC, or L4.
- The induction team will schedule the induction on EPR and arrange for the woman to be contacted to confirm the date given
- Induction of labour will be commenced on the IOL suite, level 4.
- The IOL midwife will co-ordinate the workload on the IOL suite, inviting women into hospital when a bed is available, 24 hours a day. She will

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instigate communication with women waiting at home by telephone, keeping them up to date with the situation on the IOL suite and Delivery Suite/Birth Centre.

- In patients on the induction suite should be included in the daily ward round by a senior obstetrician
- Women should be reviewed by an obstetrician prior to a second method of induction on the induction suite
- On arrival women will be assessed by the IOL midwife, to include:
 - Discussion about process, what to expect and obtain consent
 - Confirmation of dates and indication for IOL
 - Review previous USS to exclude low lying placenta
 - Abdominal palpation and ultrasound scan to confirm cephalic presentation
 - Computerised CTG
 - Baseline observations (HR, RR, BP, temperature, urinalysis) and MEOWS score
 - VE to assess the cervix and calculate the Bishop score and determine method of IOL. During this assessment palpate for cord presentation

THE MODIFIED BISHOP SCORE	0	1	2	3
Dilatation cm	<1	1-2	2-4	>4
Length cm	>4	2-4	1-2	<1
Consistency	firm	average	soft	-
Position	posterior	mid-ant	-	-
Level	-3	-2	-1; 0	+

- Insertion of cervical ripening balloon (CRB) is appropriate for women who meet the inclusion criteria and should be inserted by a midwife or doctor who has received the appropriate training. Suggested Bishop Score less than 6 however if ARM is deemed possible this should be recommended and facilitated as soon as possible.
- Administration of prostaglandins as second line where it is deemed CRB is not appropriate or not accepted by woman. For women who meet the criteria specified in the PGD, the IOL midwives may administer Propress/prostaglandin (See [Appendix 3](#))
- Do not recommend CRB or Prostaglandins if ARM is possible but there is delay in transferring to intrapartum area
- The IOL midwife will undertake both maternal and fetal observations regularly and provide analgesia and other medication as required. She will arrange transfer of women to Delivery Suite or Birth Centre (see [Planning place of birth GL887](#) for criteria) when appropriate.

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- All women undergoing induction of labour in the unit should have a fluid balance recorded. Refer to [Hyponatraemia guideline \(GL1177\)](#)

1.5 Methods of Induction of Labour

1.5.1 Cervical ripening balloon

- ◆ Induction of labour using a cervical ripening balloon is undertaken on the IOL suite, it is a silicone double balloon catheter. The maximum balloon inflation is 60ml/balloon, which should be filled with saline.
- ◆ The CRB is designed to gradually dilate the cervix and facilitate induction of labour by providing gentle and constant pressure at the level of the cervix at both the external and internal os.
- ◆ Exclusions:
 - Any contraindication to vaginal birth (placenta praevia, vasa praevia,),
 - Ruptured membranes
 - Unstable presenting part which is not in the pelvis
- ◆ The balloon should be inserted by relevant trained staff on the IOL suite or delivery suite.
- ◆ Encourage women to pass urine prior to insertion.
- ◆ Once inserted monitor bladder function. If unable to pass urine every 3-4 hours remove the balloon and offer vaginal examination to assess Bishop score. If cervix remains unfavourable discuss recommendation for Propess pessary to continue the induction.
- ◆ Undertake maternal observations and CTG twice daily, more frequently if necessary depending on reason for induction of labour
- ◆ Inform women that they may feel some abdominal discomfort 4-6 hours following insertion of the CRB and offer simple analgesia
- ◆ CRB should remain in for 12 hours before removal and assessment for ARM + oxytocin. If labour ward is busy, it may remain in situ for a maximum of 24 hours.
- ◆ There must be good communication between the induction midwife and delivery suite coordinator to ensure patient flow through the service is optimised
- ◆ If the presenting part is high after balloon removed, encourage to walk around for 60 minutes before ARM. If the presenting part remains unstable refer to a senior obstetrician for advice before undertaking ARM
- ◆ If it is not possible to perform ARM following the removal of the balloon speak to a senior obstetrician and offer Propess if appropriate

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- ◆ NB women who have had a previous caesarean section must not be given Propess without a review by a senior obstetrician
- CRB should not be used in conjunction with prostaglandins.
- If SROM occurs the balloon should be removed
- ◆ If the balloon falls out reassure the woman that it is likely to have been effective. Offer vaginal examination to confirm favourable cervix and arrange ARM and transfer.
 - ◆ If balloon falls out before 12 hours and the cervix is unfavourable, discuss findings with the woman and recommend inserting another CRB. Continue the timings from the original insertion date and time.
 - ◆ Assess for suitability for Outpatient IOL. All women who do not meet the outpatient criteria are recommended to have Inpatient IOL.
 - If returning home for outpatient IOL women must be given Date/time to return for 12 and 24-hour assessment and appropriate contact numbers. Ensure woman is happy to return home.
 - Please see [Appendix 4](#) for insertion guide.

1.5.2 Propess

- ◆ Should be second line option for induction of labour only where induction with CRB is contraindicated, staff have been unable to insert balloon, maternal preference or following unsuccessful IOL with CRB.
- ◆ Controlled release form of Dinoprostone (10mg), with an attached tape, given as a single dose lasting 24 hours
- ◆ Dinoprostone should not be used in women history of caesarean section.
- ◆ Should not be given where contractions are already present or in the presence of an abnormal CTG.
- ◆ Is inserted into the posterior fornix and rotated into a transverse position. The fingers should be removed carefully so that the tape unravels towards the vulva. Most of the tape should be placed in the lower part of the vagina, to prevent the woman accidentally pulling the propess out.
- ◆ The woman should remain lying down for at least 30 minutes following insertion.
- ◆ Observations while propess is in situ: 4 hourly HR, BP, temp, RR and auscultation of fetal heart
- ◆ Further vaginal assessment is necessary only if the woman is contracting 3-4:10 or the membranes rupture
- ◆ Once contractions begin, a CTG should be performed for fetal wellbeing and one Hourly maternal observations / FH if initial CTG Normal.

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- ◆ If pain is experienced and opioid analgesia is requested this should prompt the midwife to commence a CTG
- ◆ Remove propess if there are regular contractions and the cervix if >3cm dilated
- ◆ If SROM is confirmed, the propess may remain in-situ unless in established labour or 24 hours has lapsed
- ◆ If not removed before, the propess should be removed after 24 hours.
- **Hyperstimulation**
 - ◆ >5 uterine contractions in 10 minutes accompanied by FHR abnormalities
 - ◆ If the hyperstimulation does not settle despite removing the propess, refer to duty obstetrician for administration of Terbutaline 0.25mg subcutaneously.
- **Tachysystole**
 - ◆ >5 uterine contractions in 10 minutes with no FHR abnormalities
 - ◆ Pessary should not be removed. CTG should be observed and continued
- **If the pessary falls out**
 - ◆ If it is in the bed or the woman's pants, reinsert the same pessary as soon as possible
 - ◆ If the pessary falls down the toilet/floor, insert a new pessary as soon as possible
 - ◆ The 24 hours is still calculated from the time the first propess is inserted
- **2nd propess or CRB following after propess**
 - ◆ If unsuitable for ARM on removal of the first propess, a CRB can be used if agreed with the woman and not contraindicated (see above). Alternatively, a second propess may be inserted after discussion with medical staff

1.5.3 Outpatient induction

- ◆ Women who meet the following criteria should be encouraged to have out-patient IOL:
 - 37 completed weeks of pregnancy
 - Low risk
 - Singleton pregnancy
 - No SROM
 - Cephalic presentation and non-ballotable head

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- Unscarred uterus
- The women must understand the information leaflet and be able to follow the instructions/advice ('Induction of Labour- Advice on returning home')
- The woman must have a responsible adult who can stay at home with her
- The woman must be happy to go home and have transport to return
- ◆ Reasons to return prior to the arranged date and time:
 - Starts contracting (more than period type cramps)
 - SROM
 - Decreased fetal movements
 - Feels unwell
 - Unable to pass urine (CRB)
 - Raised temperature/flu-like symptoms
 - Worried for any reason
 - Propess or CRB falls out

1.5.4 Prostin

- ◆ Dinoprostone 1mg
- ◆ May be used where there has been failure to induce a woman after one or two propress has been given.
- ◆ May be used by the midwife (reference Prostin PGD100) to induce labour at 18-24hrs following PROM
- ◆ The patient must be discussed with a consultant before administration (exception: term PROM low risk)
- ◆ The applicator is inserted into the posterior fornix and the gel released, taking care not to insert any gel into the cervical canal
- ◆ Monitor the fetus with a CTG continuously for 1 hour following insertion or longer if necessary.
- ◆ Repeat CTG when contractions begin

1.5.5 Amniotomy

- ◆ To be performed where Bishop score >6 at the beginning of the induction or in situations in which it is deemed possible to undertake ARM
- ◆ To be performed after removal of CRB or propress
- ◆ ARM may be carried out on the IOL suite prior to transfer to delivery suite provided a midwife is available for labour care

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- ◆ If a woman is ARM'able and the head is felt to be high such that there may be an increased risk of cord prolapse, escalate to the MW in charge and the on-call obstetric team (registrar on bleep 555 or consultant on bleep 266). They may discuss either doing a controlled procedure with fundal pressure or commencing oxytocin to induce contractions.
- ◆ Oxytocin should be commenced ideally within 2 hours of ARM. However, this should be discussed with and agreed by the woman.
- ◆ Women with GBS should be prescribed IV antibiotics at the start of their induction and these should be administered when ARM (SROM) occurs.
- ◆ If contractions start following ARM assess suitability for care on the midwife led unit

1.5.6 IOL on the Midwifery led unit

- **Women who go into labour after CRB, one Propess and/or Amniotomy** may labour on Rushey MLU if they otherwise remain low risk. A CTG must be undertaken as part of a full assessment prior to transferring to the MLU for labour care
- **Low risk multiples undergoing induction of labour who are otherwise low risk may transfer to the midwifery led unit for ARM**
 - ◆ **Following ARM, the woman is encouraged to mobilise for 1 hour**
 - ◆ **Reassessment should be offered 4 hours after ARM with recommendation to transfer to delivery suite if there are no contractions or cervical changes**
 - ◆ **Low risk primips who request this pathway can be supported to do so following discussion with Rushey core midwife and maternity coordinator**
 - ◆ **In the absence of robust evidence this pathway should be audited annually**

1.6 Delay in transfer to intrapartum area⁽⁵⁾

- There must be good communication between the IOL midwife and delivery suite coordinator to facilitate timely transfer of women to the intrapartum area
- The names of women undergoing IOL should be displayed on the delivery suite including reason for induction and method

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- Once suitable for ARM (8 hours following prostin if SROM) the following rag system should be used to give a visual display on delivery suite of delays in transfer. The induction midwife is responsible for ensuring this is displayed on the delivery suite whiteboard:
 - Green - Within 12 hours of being suitable for ARM/transfer to intrapartum area
 - Amber - 12- 24 hours of being suitable for ARM/transfer to intrapartum area
 - Red – more than 24 hours waiting for ARM/transfer to intrapartum area
- Women within the red category should be escalated to the consultant obstetrician and maternity coordinator and a Datix completed
- Escalation policy to be followed
- Ensure couples are kept updated if there are any delays in transferring their care to the intrapartum area

1.7 Unsuccessful IOL

- If not able to perform ARM following above undertake a full assessment including computerised CTG
- Senior obstetric review to discuss and agree plan including further attempts, expectant management or caesarean birth

1.8 Patients declining IOL by 42 weeks (or 40 weeks for women > 40 years at conception)

- Counsel the women about the risks and benefits of declining IOL, so she is able to make an informed decision
- Ensure that there:
 - Is no medical or obstetric reason for delivery
 - Are adequate fetal movements
 - Is no IUGR and normal liquor volume (USS for growth and LV)
 - Is a reassuring CTG
- If the above are normal, arrange for the woman to have:
 - Twice weekly liquor volume
 - Alternate day CTG's
 - Weekly ANC appointments

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- Delivery should be strongly advised if there are any non-reassuring changes in fetal movements, CTG traces or a reduction in liquor volume

1. PRE-LABOUR RUPTURE OF MEMBRANES: INDUCTION OF LABOUR

2.1 Introduction

In patients with confirmed term (≥ 37 weeks) pre-labour rupture of membranes, labour is induced with the aim of minimising fetal and maternal infection.

2.2 Process

- Women with suspected SROM should be seen in the unit and the following assessment carried out:
 - Maternal observations (BP, RR, HR, temp) and MEOWS score
 - Abdominal palpation for presentation and determining how many fifths of the presenting part are palpable
 - Auscultation of fetal heart rate in low risk mothers
 - AN CTG for consultant care patients or any patient where there is concern for fetal or maternal wellbeing or where presenting part is not stable and well-applied to the cervix
 - If obvious PROM (i.e. liquor is seen), perform sterile speculum examination to exclude cord/limb presentation (avoid digital examinations due to increased infection and release of prostaglandins, which may stimulate the process)
 - If no liquor seen, perform a sterile speculum examination to confirm SROM and exclude cord or limb prolapse
 - If no liquor seen on speculum, perform an AmniSure test to confirm SROM
- Women should be given the leaflet 'Pre-labour Rupture of Membranes'
- The aim is for the baby to be born within 48 hours following SROM (at term)
- Women should be offered expectant management for 18-24 hours following SROM or the choice of proceeding with the induction ASAP. If woman opts for expectant management a vaginal examination to conduct a modified Bishops Score should **ONLY** be offered when they return to hospital.
- Women undergoing expectant management may wait at home. They should be advised to return if:
 - Liquor becomes meconium stained
 - Contractions become regular and painful

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- Maternal pyrexia (37.2)
- Change in fetal movements
- Discuss with senior obstetrician and delivery suite coordinator and aim to offer immediate induction for women with:
 - Meconium
 - Known GBS carriers
 - Pyrexia
- Start antibiotics in patients with pyrexia/ >48 hours post-ROM:
 - Oral (not in labour)
 - Amoxicillin 1g 8 hourly for 7 days or until in established labour.
 - Azithromycin 500mg 24 hourly for 7 days or until in established labour for women with penicillin allergy
 - Intravenous (in labour)
 - Benzylpenicillin 3g stat dose followed by 1.5g 4 hourly until delivered
- **IV teicoplanin 10mg/kg STAT dose and then 10mg/kg every 12hours until delivered if penicillin allergy. If GBS positive commence IV antibiotics when SROM diagnosed and advise immediate induction.**
- **If the cervix is unfavourable (suggested BS $\leq$ 6), consider administration of Propess or Prostin gel following discussion with an obstetrician.** Arrange this via the IOL suite or DAU.
- If Propess has been inserted, commence Oxytocin 24 hours later
- If Prostin has been inserted, commence Oxytocin 6-8 hours later
- When SROM is confirmed the woman's details (including phone number) must be recorded on the appropriate board on delivery suite and the DS coordinator made aware.
- Woman should be encouraged to phone delivery suite to arrange a suitable time to be admitted this will usually be between 18 and 24 hours after SROM.
- If the augmentation cannot be commenced by 24 hours, due to delivery suite capacity, the woman should be brought into MAU. She should have a full set of observations, CTG and the offer of a vaginal examination to assess bishop score. Prostin should be considered following conversation with an obstetrician.

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- If capacity remains an issue, the woman should be admitted to the postnatal ward to await augmentation. Any concerns while admitted should be escalated to the obstetric team.
- admission to antenatal ward to await augmentation.

References

1. NICE guidance NG207 <https://www.nice.org.uk/guidance/NG207>, November 2021
2. NICE guidance CG190 <https://www.nice.org.uk/guidance/cg190>, Dec 2014 updated Feb 2017 – replaced by NG235 (Sept 2023)
3. Ockenden Review <https://www.gov.uk/government/publications/final-report-of-the-ockenden-review> March 2022
4. Howell and Blott (2020) Very advanced maternal age The obstetrician and Gynaecologist <https://obgyn.onlinelibrary.wiley.com/doi/abs/10.1111/tog.12710>
5. NHS England -South East Maternity and Neonatal Team. 2024 Induction of Labour

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Appendix 2 - PGD Inclusion and Exclusion Criteria for Midwifery prescribing of Prostin and Propess

Pregnancy equal to or greater than 38 weeks' gestation

Indications for IOL where a midwife can administer Propess include:

- Post-dates pregnancies**
- Immediate induction for term pre-labour rupture of membranes**
- Reduced fetal movements
- Gestational diabetes/diabetes
- Pregnancy Induced Hypertension and/or PET
- Reduced liquor volume

Obstetric Cholestasis in the presence of the following clinical findings:

- Normal maternal observations
- Engaged, Cephalic presentation.
- Normal CTG prior to administration.
- Unfavourable cervix with a Bishop's score <6 and intact membranes (except PROM indication).
- Not in labour.
- Absence of uterine scar

***decision for induction for these indications can be made by a midwife. Decision for IOL for all other indications has to be made by an Obstetrician.*

IOL for any other indication not listed above, Propess has to be prescribed by an Obstetrician

The following situations must be discussed with an obstetrician before propess is prescribed and administered

- Previous Caesarean Section
- Regular contractions
- Preterm pregnancy
- Unengaged cephalic presentation

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Appendix 3 - RBFT Tool to inform induction of labour decision

CLINICAL CONDITION	RECOMMENDED GESTATION OF IOL (WEEKS)	PRIORITY LEVEL
POST DATES	40+7	1
PRE-EXISTING DIABETES Type 1 or 2	37+0 to 38+6	1
GDM LOW RISK Diet/Metformin /Insulin - Stable blood glucose (most readings in target) - Normal amniotic fluid volume and normal growth	40+3 - 40 + 6	2
GDM POSSIBLE COMPLICATIONS - Macrosomia (AC >95th centile and/or EFW >95th centile at 36 weeks accelerated growth from previous scans) (also consider CS) - Increased risk shoulder dystocia (previous shoulder dystocia or short maternal stature) (also consider CS) - Fetal Growth Restriction - arrange through SGA clinic	37-40	2
MATERNAL AGE 40 – 44 years old at booking 45 years old at booking	40 38	2 2
IUGR/SGA Including low CPR/Raised Dopplers Individual recommendation for IOL on delivery suite should be made by fetal medicine consultant / associate specialist only	Individual at request of consultant obstetrician	1
PET (ONLY IF INPATIENT)	Diagnosed at <37+0 Consultant decision	1
	Diagnosed at ≥37+0 weeks - induce as soon possible after diagnosis	1

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	(usually within 48 hours)	
HYPERTENSION (non-proteinuric and normal bloods) Not requiring admission	40 – 40+6	2
RAISED PCR (≥ 30)	40 – 40+6	2
RAISED PCR (≥ 30) with hypertension/PET symptoms	39-40+6	2
OBSTETRIC CHOLESTASIS (irrelevant of treatment): Bile acids > 100 (AT ANY STAGE)	37+0 – 39+6	2
PATIENTS ON FULL THERAPEUTIC ANTICOAGULATION IF MDT RECOMMENDS NOT SPONT LABOUR After discontinuing LMWH for 24 hours – to avoid repeated episodes of missing anticoagulation	39	1
APH (ONLY IF INPATIENT)	Individualise	1
↓PAPP-A / ↑AFP WITH ASSOCIATED SGA/IUGR	39 – 39+6	2
PREVIOUS TRAUMATIC BIRTH SEVERE ANXIETY	$\geq 38+3$ weeks Booked through consultant midwife clinic / ANC following discussion with consultant midwife or consultant obstetrician	Liaise with relevant clinicians or if not available DS consultant if needs to be deferred
MENTAL HEALTH	$\geq 38+3$ weeks Booked ONLY through joint perinatal mental health clinic, or consultant mw clinic after discussion with PMH practitioner and consultant obstetrician	Liaise with relevant clinicians or if not available DS consultant if needs to be deferred

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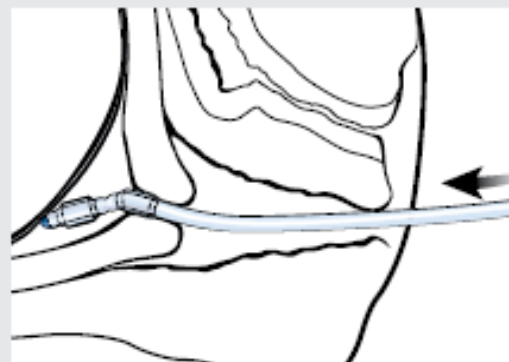
Appendix 4 - Uterine tamponade balloon insertion

- ◆ Induction of labour first line will be using a cervical ripening balloon catheter (CRB) undertaken at RBFT using a uterine tamponade balloon.
- ◆ It is a silicone double balloon catheter with maximum inflation of 60ml per balloon.
- ◆ Advantage of balloon is that it safely ripens and dilates the cervix without any pharmaceutical involvement (prostaglandins). This should mean that there is a reduction in hyperstimulation and safe to use in IUGR/SGA babies.
- ◆ Steps of insertion:

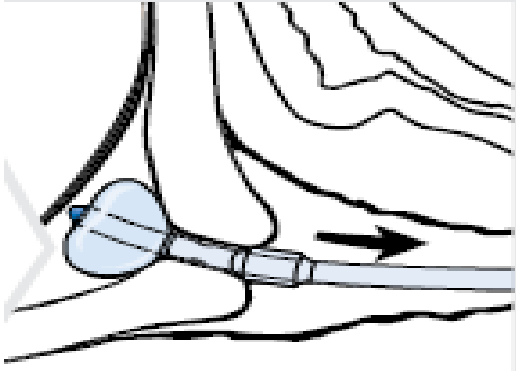
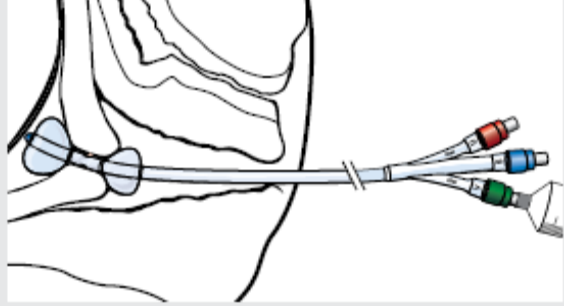
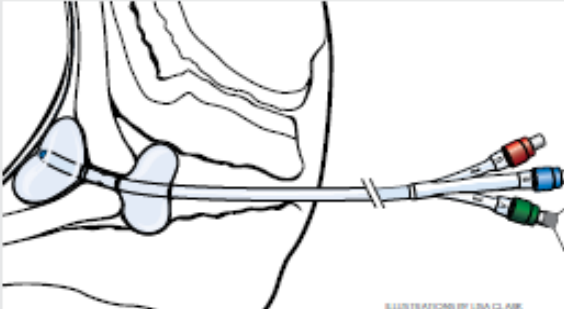


Fig. 1 – Pergo postpartum balloon

- Perform a VE. Insert two fingers into the vagina. Using the other hand advance the catheter through the cervix such that both balloons pass through the cervical canal.



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○ Inflate the first balloon through the catheter valve U (uterus) with 40mls of sterile saline. Once inflated pull the device back until the inflated balloon reaches the internal os.	
○ Once the other balloon is outside the external os inflate the vaginal balloon catheter (marked V) with a further 20ml of saline	
○ Once happy that the balloons are sited on either side of the cervix slowly fill with saline in 20ml increments to a maximum of 60ml per balloon. ○ The catheter can hang loosely.	

- ◆ As an alternative, the procedure can be done with speculum by placing the woman in the correct position or lithotomy, inserting the speculum to visualise the cervix. Passing the cervical ripening balloon through the cervix with sponge forceps (or using the stylet) and follow the above steps.
- ◆ If using the stylet, then ensure the distal tip of the stylet is in line with the distal tip of the balloon catheter. Then tighten it so the wire cannot move and place the light blue handle into the blue port labelled S. Once you have transversed, the internal os remove the stylet before advancing the catheter any further.
- ◆ Once both balloons have been inflated with the first amount (40mls of saline in the uterine balloon and 20mls in the vaginal balloon), remove the speculum.

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- ◆ Explain to the woman that she may experience some abdominal discomfort approximately 2-3 hours after insertion and offer codeine and paracetamol.
- ◆ If severe discomfort continues and simple analgesia does not relieve the discomfort, discuss whether the woman would prefer for the balloon to be removed.
- ◆ If unable to pass urine, remove the balloon to ensure bladder function resumes. Offer vaginal examination to assess Bishop score and if cervix remains unfavourable discuss recommendation for Propess pessary to continue the induction.

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